Spatial Analysis

June 4, 2023

1 Methods

Issues

- Need to review length
  - For LS, epi is a lot wider than dermis. How do we draw the length?
  - DD intact, length correct? CD3 peak seems too high in Fig 1
- For JM, region 2 is excluded from analysis.
- Need to review segmentation. flow .5 seems to results in many more cells than flow .4, e.g. for CD11c in DD

For each sample, we will compare the distribution of each markers in the normal and lesion regions.

- Markers for DD: CD3, CD4, CD8a, CD11c, CD15, CD45, CD68, CD69, CD123, CD279
- Markers for CF: CD1a, CD3, CD4, CD8a, CD11c, CD15, CD45, CD69, CD123, CD279 (CD68 did not pass QC)

For CD123, segmentation is done at flow=0.5 to get more masks in the bright area and a threshold of 5000 is set for total intensity to remove masks in the debris field in the lesion area.

The size of the skin tissue lesion area is determined by morphology using wheat germ agglutinin (WGA) flourescent stain. ImageJ is used to demarcate the lesion area, then a straight line ROI is drawn to span from one side of the lesion to the other side of the lesion, with the endpoints of the line positioned at the outermost skin boundary.

Some considerations on region boundary and distance metric:
1. Should we draw the boundary between normal and lesion based on WGA only or all markers?
2. Shall each ROI be convex?

For (1), the two choices lead are answering different questions. Drawing boundary based on WGA only answers a stronger question than drawing the boundary based on all markers.

For (2), a convex shape is a shape that a line connecting any two points in the shape completely lies within the shape. Practically speaking, in this context it is equivalent to drawing a straight line to separate normal from lesion. Pros for straight boundary: the span between any cell and the closest point on the skin lies completely within the region. So the distance measure is well defined. In contrast, if a line from a cell to its closest point on the skin surface cuts through both normal and lesion regions, the interpretation of distance to skin surface is muddled. Pros for curved boundary: Regions (which is a type of neighborhoods) do not necessarily have geometric shapes.

These two questions are also intertwined. If we use WGA only to draw boundaries, we can only use straight line boundaries because the difference between normal and lesion is only visible in epidermis in WGA staining. A straight line can be drawn in epidermis and extend to dermis, but not a curve.

2 Results

	Counts				Counts/mm2			
	normal		lesion		normal		lesion	
	epi	dermis	epi	dermis	epi	dermis	epi	dermis
CD11c	333	2295	8194	20682	2841	5398	6301	5364
CD123	131	879	1350	8038	1117	2067	1038	2084
CD15	6	147	4449	1918	51	345	3421	497
CD279	9	346	68	2168	76	813	52	562
CD3	59	1134	92	4720	503	2667	70	1224
CD4	140	1721	245	9286	1194	4048	188	2408
CD45	341	2540	6082	15965	2909	5975	4677	4140
CD68	112	619	1305	7910	955	1456	1003	2051
CD69	17	245	27	2100	145	576	20	544
CD8a	35	639	67	3394	298	1503	51	880
area	0.12	0.43	1.30	3.86				

Table 1: DD

	Counts				Counts/mm2			
	normal		lesion		normal		lesion	
	epi	dermis	epi	dermis	epi	dermis	epi	dermis
CD11c	597	8839	1412	4600	1380	4690	3494	4555
CD123	76	3680	324	1298	175	1952	801	1285
CD15	4	1614	556	302	9	856	1376	299
CD1a	70	28	11	48	161	14	27	47
CD279	110	1305	46	852	254	692	113	843
CD3	381	7847	290	5424	881	4163	717	5371
CD4	351	6876	281	3999	811	3648	695	3960
CD45	536	9636	894	6015	1239	5113	2212	5956
CD8a	213	4280	191	3287	492	2271	472	3255
area	0.43	1.88	0.40	1.01				

Table 2: CF

	DD	CF	JM	CF/DD	JM/DD
tissue width	2861	1202	1154	0.42	0.40
CD3 to CD15 (dist2skin)	311	211	529	0.68	1.70
CD3 to CD15 (dist2DEJ)	207	166	477	0.80	2.30
CD3 epi	70	717	241	10.24	3.44
CD3 dermis	1224	5371	2561	4.39	2.09
CD3 e/d	1.17	0.77	0.62	0.66	0.53
CD15 epi	3421	1376	1393	0.40	0.41
CD15 dermis	497	299	270	0.60	0.54
CD15 e/d	0.50	0.62	0.42	1.24	0.84
CD123 epi	1038	801	1257	0.77	1.21
CD123 dermis	2084	1285	3013	0.62	1.45
CD123 e/d	6.88	4.60	5.16	0.67	0.75
CD11c epi	6301	3494	2253	0.55	0.36
CD11c dermis	5364	4555	3651	0.85	0.68
CD11c e/d	0.09	0.57	0.08	6.33	0.89

Table 3: Ulcerative. The cell counts are per mm2. The widths and seperation are measured in um.

	DD	CF	JM	CF/DD	JM/DD
tissue width	251	1531	2114	6.10	8.42
CD3 to CD15 (dist2skin)	51	-82	140	-1.61	2.75
CD3 to CD15 (dist2DEJ)	52	-22	23	-0.42	0.44
CD3 epi	503	881	675	1.75	1.34
CD3 dermis	2667	4163	3883	1.56	1.46
CD3 e/d	0.53	0.29	0.57	0.55	1.08
CD15 epi	51	9	16	0.18	0.31
CD15 dermis	345	856	88	2.48	0.26
CD15 e/d	0.54	0.09	0.43	0.17	0.80
CD123 epi	1117	175	749	0.16	0.67
CD123 dermis	2067	1952	1743	0.94	0.84
CD123 e/d	0.15	0.01	0.18	0.07	1.20
CD11c epi	2841	1380	1666	0.49	0.59
CD11c dermis	5398	4690	2945	0.87	0.55
CD11c e/d	0.09	11.50	0.30	127.78	3.33

Table 4: Intact. The cell counts are per mm2. The widths and seperation are measured in um.

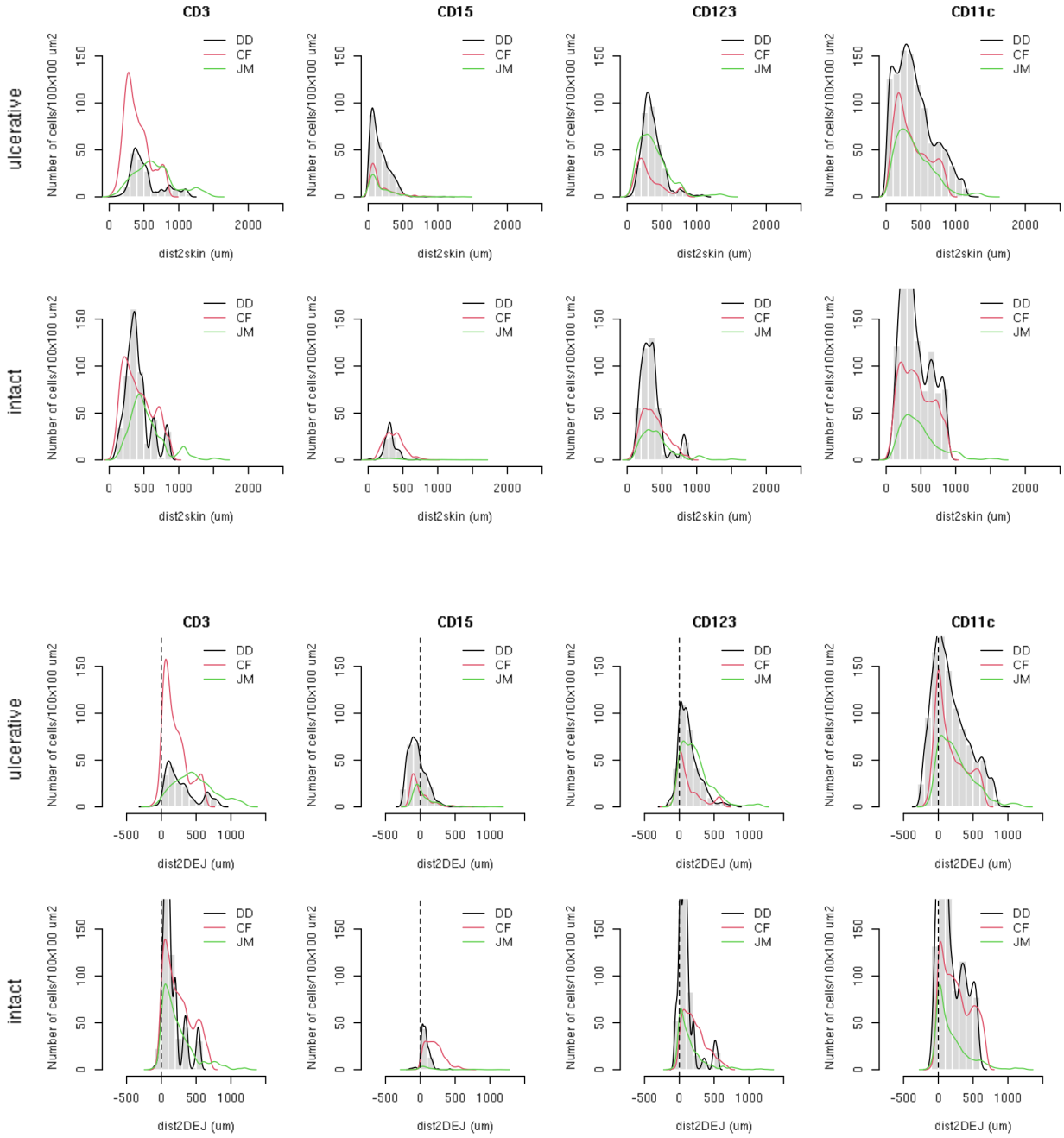


Figure 1: Number of cells per 100x100 um2 tissue as a function of distance to DEJ or skin. The curves are smoothed over histograms and each bin in the histogram represents a 100 um interval. Adding up the counts from all the bins gives the number of cells in tissue of 100 um in width. For example, CD123 epi count 1038/2862*100=36.

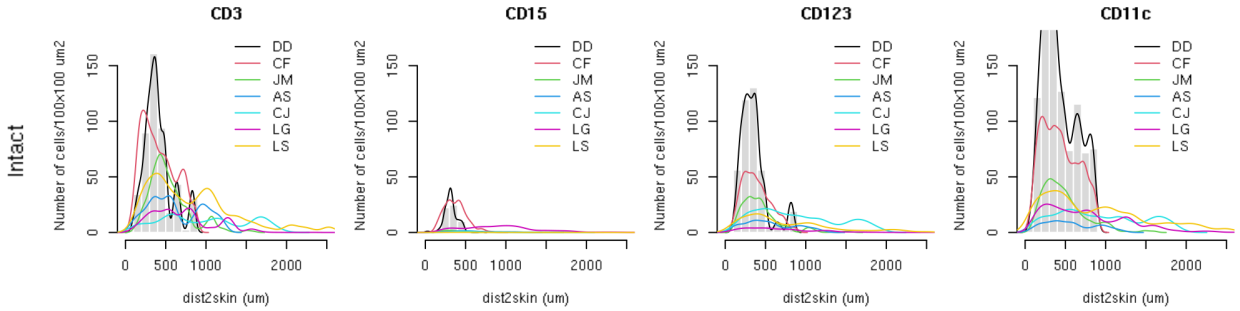


Figure 2: Number of cells per 100x100 um2 tissue as a function of distance to DEJ or skin. The curves are smoothed over histograms and each bin in the histogram represents a 100 um interval. Adding up the counts from all the bins gives the number of cells in tissue of 100 um in width.

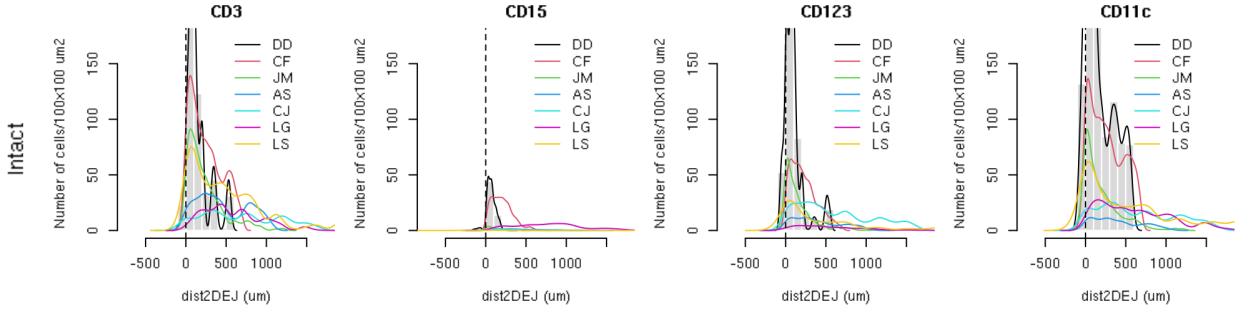


Figure 3: Number of cells per 100x100 um2 tissue as a function of distance to DEJ or skin. The curves are smoothed over histograms and each bin in the histogram represents a 100 um interval. Adding up the counts from all the bins gives the number of cells in tissue of 100 um in width.

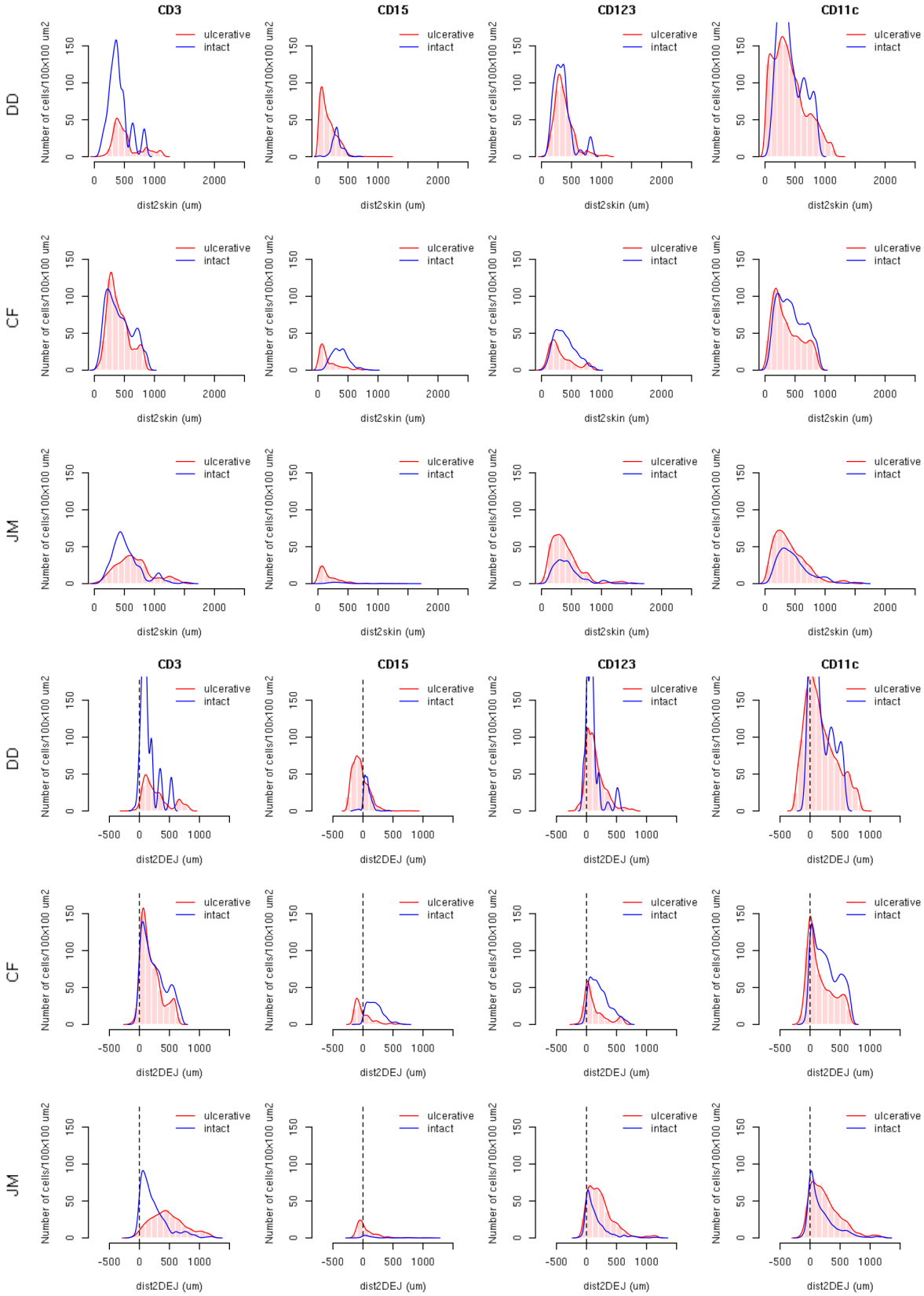


Figure 4: Number of cells per 100x100 um2 tissue as a function of distance to skin or DEJ. The curves are smoothed over histograms and each bin in the histogram represents a 100 um interval. Adding up the counts from all the bins gives the number of cells in tissue of 100 um in width.

Comparing the first two rows in the first column, we see that the ratio of the area under the curve across both epi and dermis between red and blue(ulcerative and intact) is about 1 for CD3/CF and 1/3 for CD3/DD. This suggests that CD3 is depleted in lesion in DD, but not in CF.

Compare lesion with normal in CD15/CF, we see that the lesion peak is shifted to the left compared to the normal peak. This pattern is more clear in CF than in DD. We don't see this pattern for CD3.

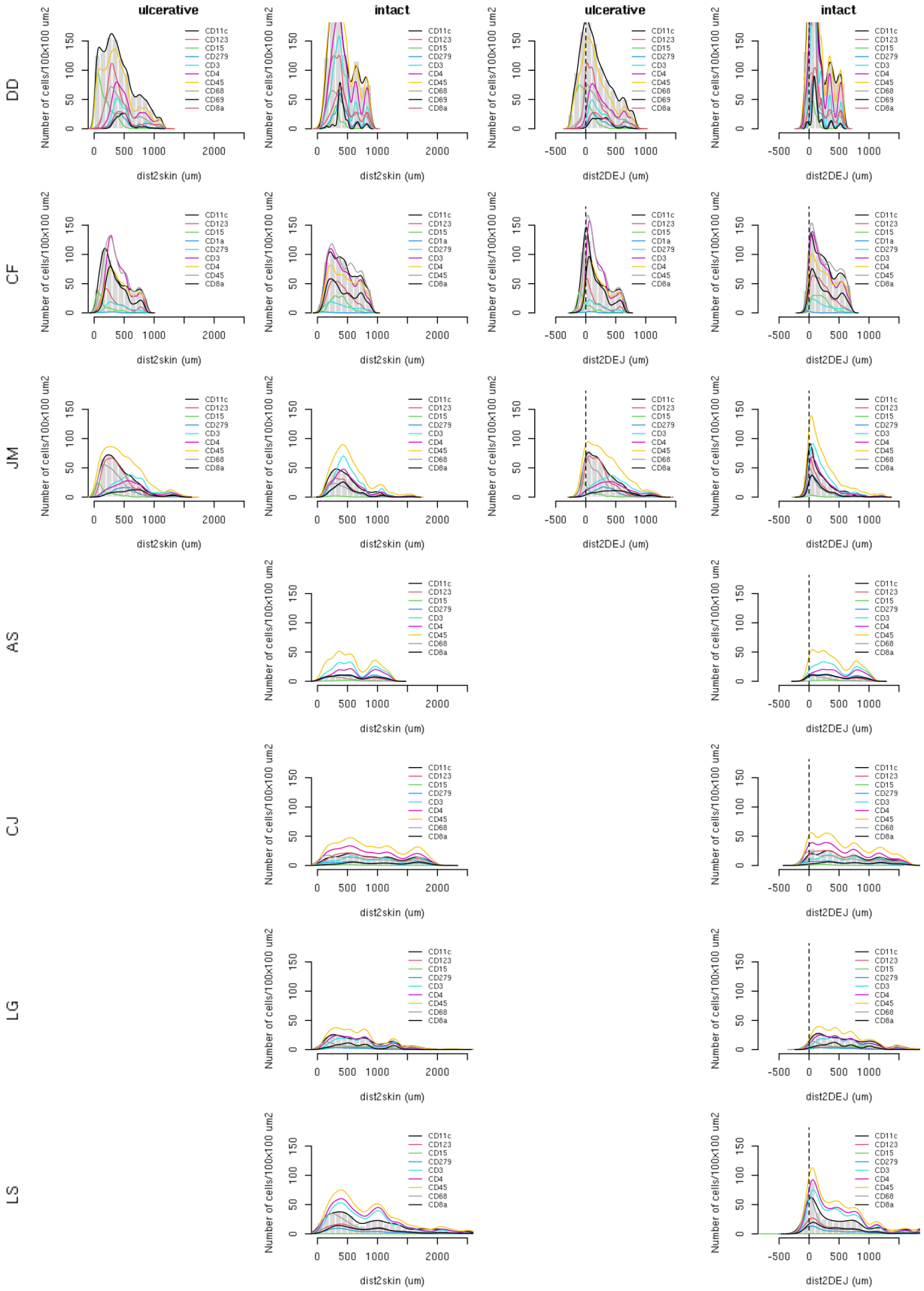
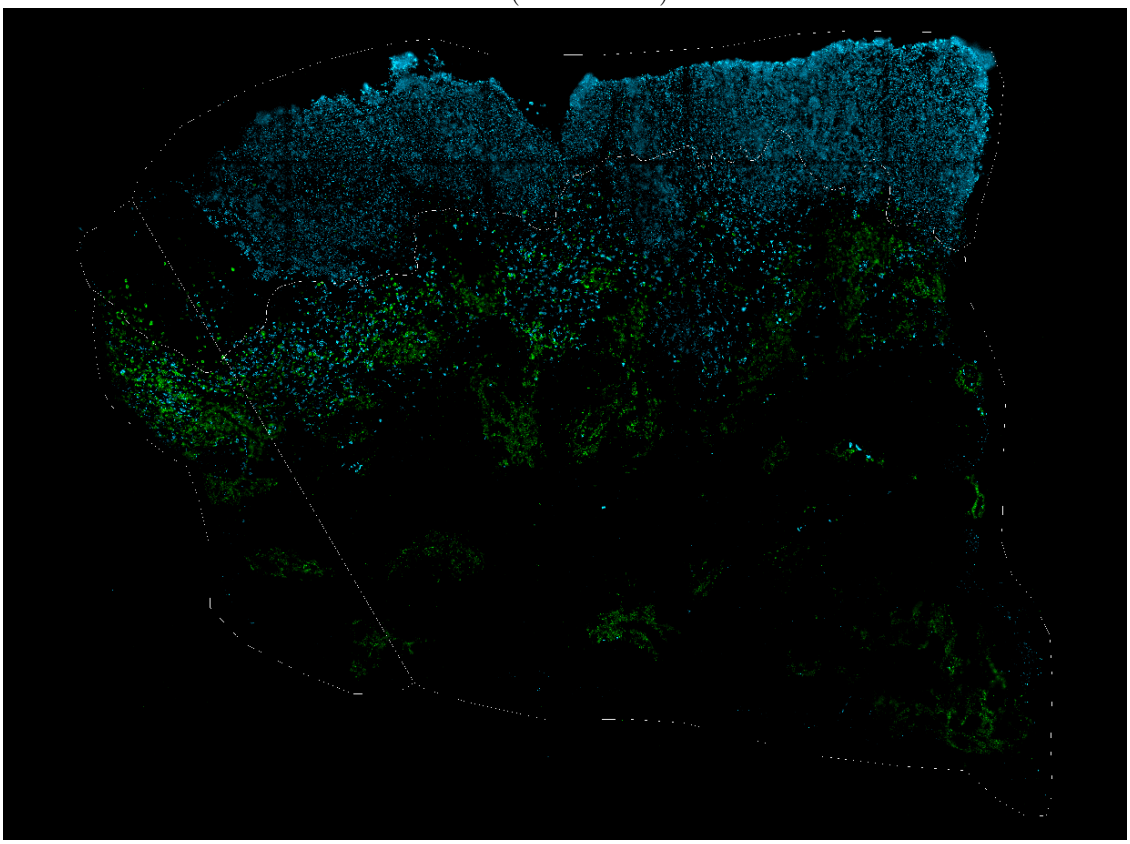


Figure 5: Number of cells per 100x100 um2 tissue as a function of distance to skin. The curves are smoothed over histograms and each bin in the histogram represents a 100 um interval. Adding up the counts from all the bins gives the number of cells in tissue of 100 um in width.

That the CD15 dist2skin peak in DD/lesion is near skin while the corresponding dist2DEJ peak is near DEJ is due to the fact that the DEJ line is very curvy.

DD (6540 × 5262)



CF (5296 × 4040)

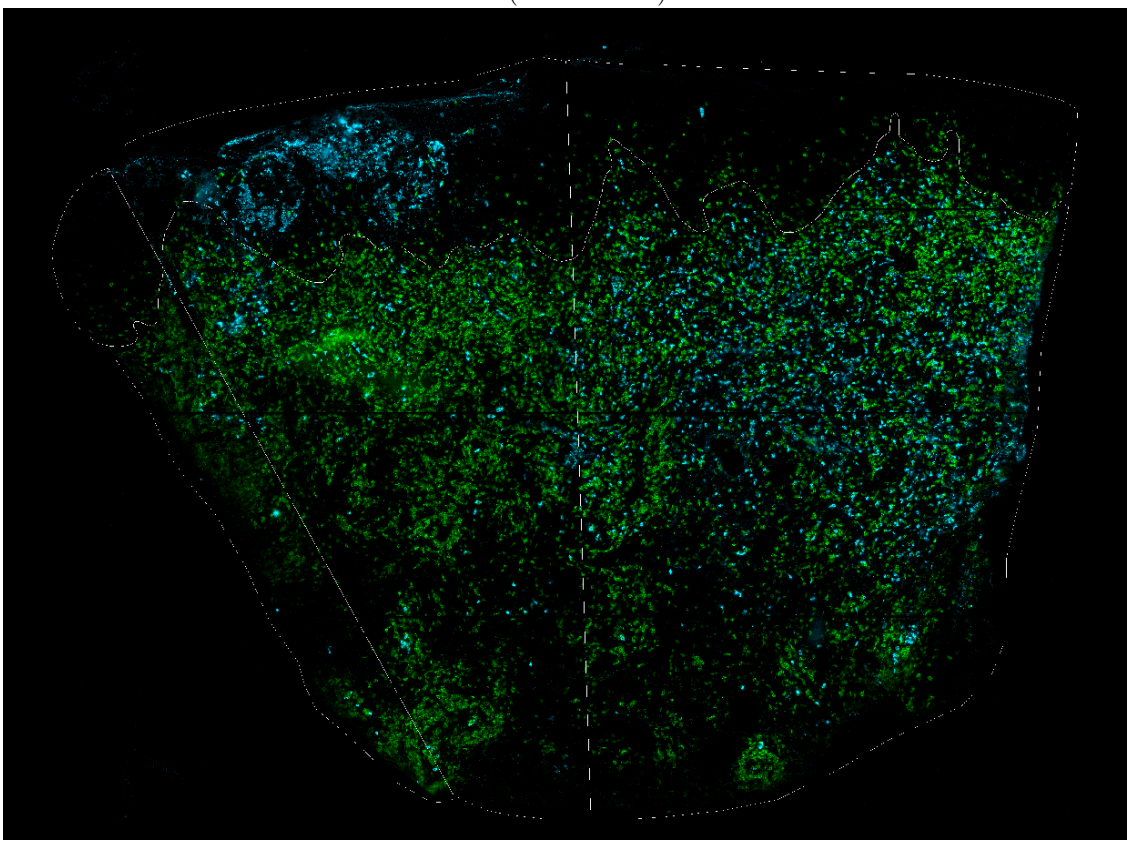


Figure 6: CD3: green, contrast limits 0-0.5; CD15: cyan. The intact/ulcerative boundary and the epidmeris/dermis boundary are shown in white. Outer boundaries are not tight.

JM (6528 × 4008)

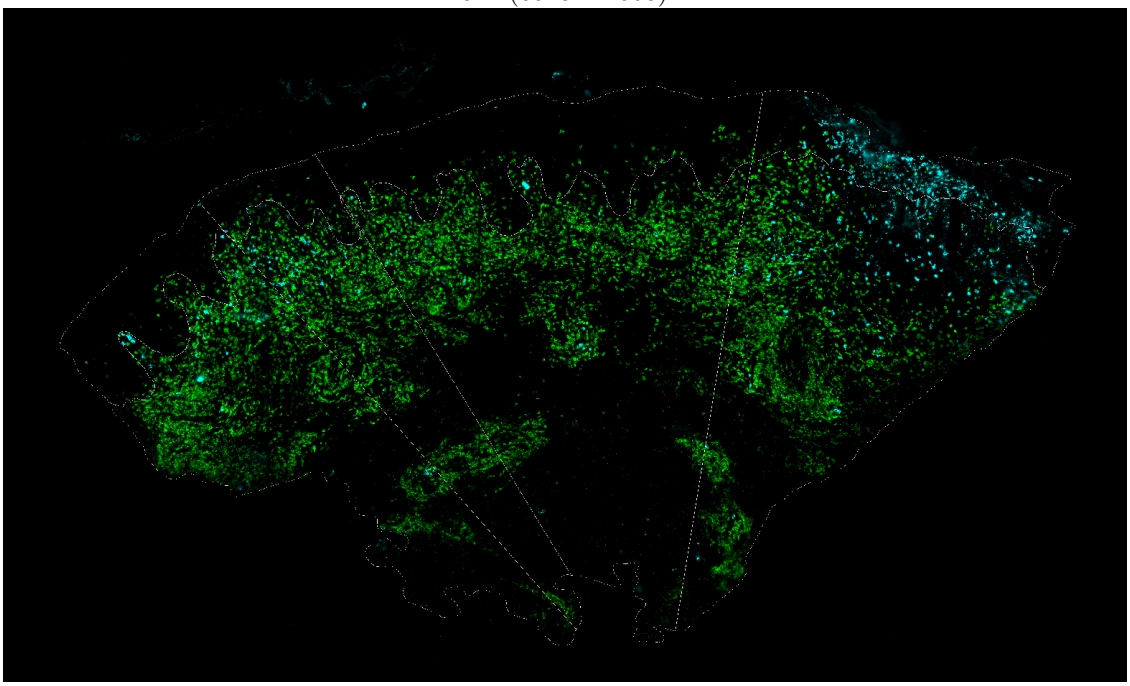


Figure 7: CD3: green; CD15: cyan, contrast limits 0-0.5. Region boudaries are shown in white.

In Figure 7 we see that DD lesion has less CD3 and is further away from DEJ than CF. Correspondingly, in Figure 1, we see that in the dermis region close to DEJ, the count density is about 50 for CF/ulcerative, CF/intact, and CF/ulcerative; but for DD/ulcerative, the count density is much lower.

Biospies with only intact regions seem to have lower number of immune cells.

3 Discussion

Neutraphil to lymphocyte ratio (NLR) in blood is a prognostic factor for cancer. Lower is better.

4 References

Chan A, Bentzen S, Rout A, Miller K. Predicting if lung cancer will relapse-the role of neutrophil/lymphocyte ratio. Hematol Oncol Stem Cell Ther. 2023;16(2):158–61.

Costa T, Nogueiro J, Ribeiro D, Viegas P, Santos-Sousa H. Impact of serum albumin concentration and neutrophil-lymphocyte ratio score on gastric cancer prognosis. Langenbecks Arch Surg. 2023;408(1):57.

Chen W, Xin S, Xu B. Value research of NLR, PLR, and RDW in prognostic assessment of patients with colorectal cancer. J Healthcare Eng. 2022;16(2022):7971415.